

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$31,317,052
Prenatal arm	\$9,867,016
Postnatal arm	\$21,450,035
Administrative load (8%)	\$2,505,364
Appropriation total (benefit + admin)	\$33,822,416
Sampling 90% CI	\$23,501,385 - \$39,132,718
Assumption band	\$18,790,231 - \$39,929,837

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$31,317,052	\$33,822,416	13,728
Universal · 6-mo postnatal	\$4,500	\$53,327,865	\$57,594,094	23,377
Universal · 12-mo postnatal	\$7,500	\$89,853,799	\$97,042,103	23,377

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$21,450,035
Total cost (12-month design)	\$52,767,087

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

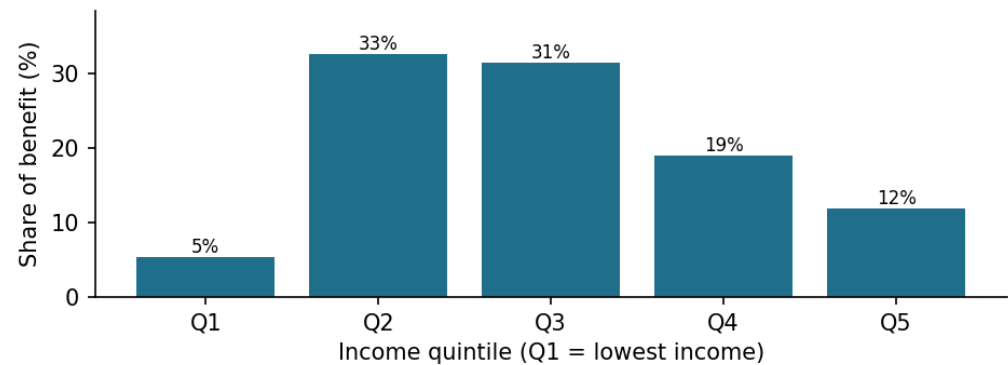
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$12,991,174
Prenatal / Postnatal	\$5,344,634 / \$7,646,540
Postnatal deferred to next FY	\$3,972,229
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$20,323,909 / \$12,991,174 / \$8,660,783

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	13,728
Expected pregnancies / infants	6,578 / 7,150
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	722	\$1,647,671	5.3%
Q2	\$57,026	110,263	4,481	\$10,221,201	32.6%
Q3	\$96,722	110,317	4,313	\$9,837,998	31.4%
Q4	\$154,809	110,231	2,594	\$5,916,636	18.9%
Q5	\$334,087	110,343	1,619	\$3,693,545	11.8%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$4,533,949	\$1,428,505	\$3,105,445	1,987
Honolulu	\$22,915,140	\$7,219,839	\$15,695,301	10,045
Kauai	\$2,371,759	\$747,267	\$1,624,493	1,040
Maui	\$1,496,203	\$471,407	\$1,024,797	656

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.